

SECTION 1: Airway & Breathing (Questions 1-20)

1. A 45-year-old patient is in severe respiratory distress with stridor. The most appropriate initial intervention is:

- A. Administer nebulized albuterol
- B. Perform endotracheal intubation
- C. Provide high-flow oxygen via NRB
- D. Insert an oropharyngeal airway

Answer: C - High-flow oxygen is the first step for a patient in respiratory distress with stridor.

2. The normal respiratory rate for an adult at rest is:

- A. 8-12 breaths/min
- B. 12-20 breaths/min
- C. 20-30 breaths/min
- D. 30-40 breaths/min

Answer: B - Normal adult respiratory rate is 12-20 breaths per minute.

3. Which of the following is a sign of adequate breathing?

- A. Use of accessory muscles
- B. Nasal flaring
- C. Equal chest rise and fall
- D. Cyanosis

Answer: C - Equal chest rise and fall indicates bilateral lung expansion and adequate ventilation.

4. A patient with a GCS of 8 requires immediate:

- A. Administration of oral glucose
- B. Airway management
- C. Splinting of extremity fractures
- D. Application of a cervical collar

Answer: B - A GCS ≤ 8 indicates a patient cannot protect their own airway and requires immediate airway management.

5. The most common cause of airway obstruction in an unconscious patient is:

- A. Foreign body
- B. The tongue
- C. Laryngeal edema
- D. Vomitus

Answer: B - The tongue is the most common cause of airway obstruction in unconscious patients due to loss of muscle tone.

6. When using a bag-valve mask, the correct ventilation rate for an adult with a pulse is:

- A. 6-8 breaths/min
- B. 10-12 breaths/min
- C. 15-20 breaths/min
- D. 20-24 breaths/min

Answer: B - For an adult with a pulse, ventilate at 10-12 breaths per minute (one breath every 5-6 seconds).

7. Which finding suggests a tension pneumothorax?

- A. Bilateral breath sounds
- B. Tracheal deviation toward the affected side
- C. JVD and hypotension
- D. Wheezing on auscultation

Answer: C - Tension pneumothorax presents with JVD, hypotension, tracheal deviation AWAY from the affected side.

8. The correct compression-to-ventilation ratio for adult CPR with an advanced airway is:

- A. 15:2
- B. 30:2
- C. Continuous compressions with 1 breath every 6 seconds
- D. 5:1

Answer: C - With an advanced airway, provide continuous compressions with one breath every 6 seconds (10 breaths/min).

9. A patient presents with pink, frothy sputum and severe dyspnea. This is most consistent with:

- A. Pneumonia
- B. Pulmonary embolism
- C. Pulmonary edema

D. Asthma exacerbation

Answer: C - Pink, frothy sputum is a classic sign of pulmonary edema, often associated with CHF.

10. The appropriate size oropharyngeal airway for an adult is measured from:

- A. Corner of the mouth to the earlobe
- B. Center of the mouth to the angle of the jaw
- C. Nose to the earlobe
- D. Chin to the sternum

Answer: A - An OPA is measured from the corner of the mouth to the earlobe or angle of the jaw.

11. Which of the following is NOT a sign of respiratory failure?

- A. SpO₂ < 94%
- B. Respiratory rate of 8
- C. Cyanosis
- D. Respiratory rate of 28

Answer: B - A respiratory rate of 8 is bradypneic but not necessarily respiratory failure if oxygenation is adequate.

12. The most effective method to open an airway in a trauma patient is:

- A. Head-tilt chin-lift
- B. Jaw-thrust maneuver
- C. Chin-lift only
- D. Neck extension

Answer: B - The jaw-thrust maneuver is the preferred method for opening the airway in trauma patients.

13. A patient with COPD is showing signs of respiratory fatigue. The most appropriate intervention is:

- A. Administer 100% oxygen via NRB
- B. Assist ventilations with BVM
- C. Administer epinephrine
- D. Place in Trendelenburg position

Answer: B - A fatiguing COPD patient needs ventilatory support with a BVM to prevent respiratory arrest.

14. The normal SpO₂ range for a healthy adult is:

- A. 85-90%
- B. 90-94%
- C. 95-100%
- D. 100% only

Answer: C - Normal SpO₂ for a healthy adult is 95-100%. Below 94% is considered hypoxemic.

15. Which medication is used to treat anaphylaxis?

- A. Albuterol
- B. Epinephrine
- C. Atropine
- D. Nitroglycerin

Answer: B - Epinephrine is the first-line treatment for anaphylaxis.

16. A patient with a stoma requires suctioning. The maximum suction time for an adult is:

- A. 5 seconds
- B. 10 seconds
- C. 15 seconds
- D. 30 seconds

Answer: C - Maximum suction time for an adult is 15 seconds to prevent hypoxia and mucosal damage.

17. The correct depth of chest compressions for an adult is:

- A. At least 1 inch
- B. At least 2 inches
- C. At least 3 inches
- D. 1.5 inches

Answer: B - Adult chest compressions should be at least 2 inches deep but no more than 2.4 inches.

18. Which of the following indicates effective CPR?

- A. Palpable femoral pulse during compressions
- B. Pupil dilation
- C. Absent breath sounds
- D. Hypotension

Answer: A - A palpable pulse during compressions indicates effective cardiac output.

19. A patient presents with sudden onset of difficulty breathing and chest pain after a long flight. The most likely diagnosis is:

- A. Myocardial infarction
- B. Pulmonary embolism
- C. Pneumothorax
- D. Asthma

Answer: B - Sudden dyspnea and chest pain after prolonged immobility is classic for pulmonary embolism from DVT.

20. The first step in managing a patient with a foreign body airway obstruction who is conscious is:

- A. Perform abdominal thrusts
- B. Encourage coughing
- C. Perform back blows
- D. Suction the airway

Answer: B - If the patient can cough, encourage coughing. Only intervene with abdominal thrusts if the patient cannot cough, speak, or breathe.

NREMT PRACTICE EXAM 1

Cardiovascular Emergencies (Questions 21-40)

SECTION 2: Cardiovascular Emergencies (Questions 21-40)

21. A 58-year-old male presents with crushing chest pain radiating to his left arm, diaphoresis, and nausea. The most likely diagnosis is:

- A. Gastroesophageal reflux
- B. Acute myocardial infarction
- C. Pneumonia
- D. Costochondritis

Answer: B - Crushing chest pain with radiation to the left arm, diaphoresis, and nausea are classic signs of acute MI.

22. The correct dose of aspirin for a suspected acute MI is:

- A. 81 mg
- B. 162 mg
- C. 325 mg chewable
- D. 650 mg

Answer: C - 325 mg chewable aspirin is the standard dose for suspected acute MI.

23. A patient presents with a heart rate of 180 bpm, regular rhythm, and a narrow QRS complex. This is most consistent with:

- A. Ventricular tachycardia
- B. Supraventricular tachycardia
- C. Sinus tachycardia
- D. Atrial fibrillation

Answer: B - SVT presents with a regular narrow complex tachycardia, typically 150-250 bpm.

24. The treatment for a stable patient with SVT includes:

- A. Synchronized cardioversion
- B. Vagal maneuvers
- C. Amiodarone 300mg IV
- D. Atropine 0.5mg IV

Answer: B - Vagal maneuvers (Valsalva, carotid massage) are the first-line treatment for stable SVT.

25. A patient with CHF exacerbation presents with severe dyspnea, JVD, and bilateral crackles. The most appropriate position is:

- A. Supine
- B. Left lateral recumbent
- C. Sitting upright
- D. Trendelenburg

Answer: C - Sitting upright (high Fowlers) reduces venous return and improves breathing in CHF patients.

26. Which of the following is a contraindication for nitroglycerin administration?

- A. Systolic BP > 100 mmHg
- B. Recent use of PDE5 inhibitors (Viagra)
- C. Heart rate > 100 bpm
- D. Age > 65

Answer: B - PDE5 inhibitors within 24-48 hours are an absolute contraindication for nitroglycerin.

27. The normal systolic blood pressure range for an adult is:

- A. 60-80 mmHg
- B. 90-120 mmHg
- C. 120-140 mmHg
- D. 140-160 mmHg

Answer: B - Normal systolic BP is 90-120 mmHg. Hypotension is < 90 mmHg systolic.

28. A patient presents with sudden cardiac arrest. The most common initial rhythm is:

- A. Asystole
- B. Pulseless electrical activity
- C. Ventricular fibrillation
- D. Sinus bradycardia

Answer: C - VFib is the most common initial rhythm in sudden cardiac arrest.

29. During CPR, epinephrine should be administered every:

- A. 1-2 minutes
- B. 3-5 minutes
- C. 5-10 minutes
- D. 10-15 minutes

Answer: B - Epinephrine 1mg IV/IO should be administered every 3-5 minutes during cardiac arrest.

30. A patient presents with chest pain and ST elevation in leads II, III, and aVF. This indicates:

- A. Anterior wall MI
- B. Inferior wall MI
- C. Lateral wall MI
- D. Posterior wall MI

Answer: B - ST elevation in II, III, and aVF indicates an inferior wall MI.

31. The correct dose of nitroglycerin for chest pain is:

- A. 0.4 mg SL
- B. 1.0 mg SL
- C. 4.0 mg SL
- D. 10 mg IV

Answer: A - Nitroglycerin 0.4 mg sublingual is the standard dose for chest pain.

32. A patient with a heart rate of 40 bpm and hypotension should receive:

- A. Atropine 0.5mg IV
- B. Adenosine 6mg IV
- C. Amiodarone 150mg IV
- D. Lidocaine 1mg/kg IV

Answer: A - Atropine 0.5mg IV is the first-line treatment for symptomatic bradycardia.

33. Which of the following is NOT a sign of cardiogenic shock?

- A. Hypotension
- B. JVD
- C. Warm, dry skin
- D. Altered mental status

Answer: C - Cardiogenic shock presents with cool, clammy skin. Warm skin suggests distributive shock.

34. The Cincinnati Prehospital Stroke Scale assesses all EXCEPT:

- A. Facial droop
- B. Arm drift
- C. Speech
- D. Pupil response

Answer: D - The Cincinnati Stroke Scale assesses facial droop, arm drift, and speech.

35. A patient presents with unilateral weakness, slurred speech, and a facial droop. The most time-critical intervention is:

- A. Administer aspirin
- B. Obtain a 12-lead ECG
- C. Rapid transport to a stroke center
- D. Check blood glucose

Answer: C - Time is brain. Rapid transport to a stroke center is the priority.

36. The maximum time from symptom onset for tPA administration in stroke is:

- A. 1 hour
- B. 3 hours
- C. 4.5 hours
- D. 6 hours

Answer: C - tPA must be administered within 4.5 hours of symptom onset for ischemic stroke.

37. A patient presents with severe headache, photophobia, and neck stiffness. This is most consistent with:

- A. Migraine
- B. Meningitis
- C. Tension headache
- D. Cluster headache

Answer: B - The classic triad of headache, photophobia, and neck stiffness suggests meningitis.

38. Which of the following is a late sign of increased intracranial pressure?

- A. Headache
- B. Nausea and vomiting
- C. Cushing's triad

D. Altered mental status

Answer: C - Cushings triad (hypertension, bradycardia, irregular respirations) is a late sign of increased ICP.

39. A patient with a seizure that lasts more than 5 minutes is in:

- A. Status epilepticus
- B. Cluster seizures
- C. Psychogenic seizure
- D. Simple partial seizure

Answer: A - Status epilepticus is defined as a seizure lasting >5 minutes.

40. The first-line treatment for status epilepticus in the field is:

- A. Lorazepam (Ativan)
- B. Phenytoin (Dilantin)
- C. Levetiracetam (Keppra)
- D. Phenobarbital

Answer: A - Benzodiazepines are the first-line treatment for status epilepticus.

NREMT PRACTICE EXAM 1

Medical Emergencies (Questions 41-60)

SECTION 3: Medical Emergencies (Questions 41-60)

41. A diabetic patient is found unresponsive with cold, clammy skin and a blood glucose of 45 mg/dL. The treatment is:

- A. Administer insulin
- B. Administer oral glucose
- C. Start an IV and administer D50
- D. Provide high-flow oxygen

Answer: C - An unresponsive hypoglycemic patient requires IV D50 to rapidly raise blood glucose.

42. Which of the following is a sign of an allergic reaction?

- A. Bradycardia
- B. Bronchodilation
- C. Urticaria
- D. Hypertension

Answer: C - Urticaria (hives) is a common sign of an allergic reaction.

43. A patient presents with severe abdominal pain, distension, and vomiting. The most likely diagnosis is:

- A. Appendicitis
- B. Bowel obstruction
- C. Cholecystitis
- D. Pancreatitis

Answer: B - The triad of abdominal pain, distension, and vomiting is classic for bowel obstruction.

44. The correct treatment for a patient with a suspected opioid overdose who is not breathing is:

- A. Administer naloxone first
- B. Ventilate with BVM first
- C. Start chest compressions
- D. Administer flumazenil

Answer: B - Ventilate the patient first with a BVM, then administer naloxone.

45. A patient presents with fever, tachycardia, tachypnea, and altered mental status. This is most consistent with:

- A. Sepsis
- B. Meningitis
- C. Pneumonia
- D. UTI

Answer: A - Sepsis presents with systemic inflammatory response.

46. Which of the following is a sign of anaphylaxis?

- A. Localized hives only
- B. Hypotension and wheezing
- C. Isolated facial swelling
- D. Mild itching

Answer: B - Anaphylaxis involves multi-system reactions including respiratory and cardiovascular.

47. The correct dose of epinephrine for anaphylaxis via auto-injector is:

- A. 0.15 mg IM for adults
- B. 0.3 mg IM for adults
- C. 1.0 mg IM for adults
- D. 0.5 mg IV for adults

Answer: B - The adult epinephrine auto-injector delivers 0.3 mg IM.

48. A patient presents with chest pain that worsens with deep breathing and improves when leaning forward. This is most consistent with:

- A. Myocardial infarction
- B. Pericarditis
- C. Pneumothorax
- D. Pulmonary embolism

Answer: B - Pericarditis pain typically worsens with deep breathing and improves when sitting up.

49. Which of the following is a contraindication for activated charcoal?

- A. Ingestion within 1 hour

- B. Altered mental status
- C. Acetaminophen overdose
- D. Food poisoning

Answer: B - Activated charcoal is contraindicated in patients with altered mental status.

50. A patient presents with signs of hypothermia. The core body temperature is likely below:

- A. 98.6 degF (37 degC)
- B. 95 degF (35 degC)
- C. 90 degF (32 degC)
- D. 85 degF (29 degC)

Answer: B - Hypothermia is defined as a core body temperature below 95 degF (35 degC).

51. The correct treatment for a patient with severe hypothermia and a pulse is:

- A. Active rewarming with hot water
- B. Gentle handling and passive rewarming
- C. Immediate CPR
- D. Administer warm IV fluids only

Answer: B - Severe hypothermia patients should be handled gently to prevent VFib.

52. A patient presents with hot, dry skin, confusion, and a temperature of 105 degF. This is most consistent with:

- A. Heat exhaustion
- B. Heat stroke
- C. Meningitis
- D. Sepsis

Answer: B - Heat stroke presents with hot/dry skin, altered mental status, and core temperature >104 degF.

53. The priority treatment for heat stroke is:

- A. Administer IV fluids
- B. Rapid cooling
- C. Administer antipyretics
- D. Provide oxygen

Answer: B - Rapid cooling is the priority for heat stroke.

54. A patient presents with signs of a venomous snake bite. The most appropriate treatment is:

- A. Apply a tourniquet
- B. Apply ice to the bite
- C. Keep the extremity below heart level and transport rapidly
- D. Attempt to suck out the venom

Answer: C - Keep the affected extremity below heart level and transport rapidly.

55. Which of the following is a sign of acute alcohol withdrawal?

- A. Hypothermia
- B. Bradycardia
- C. Tremors and agitation
- D. Hypotension

Answer: C - Alcohol withdrawal presents with tremors, agitation, tachycardia, and hypertension.

56. A patient presents with a blood glucose of 450 mg/dL, Kussmaul respirations, and fruity breath. This is most consistent with:

- A. Hypoglycemia
- B. Diabetic ketoacidosis
- C. Hyperosmolar hyperglycemic state
- D. Addisonian crisis

Answer: B - DKA presents with hyperglycemia, Kussmaul respirations, and fruity breath.

57. The most important initial treatment for DKA in the field is:

- A. Administer insulin
- B. Administer sodium bicarbonate
- C. Fluid resuscitation
- D. Administer potassium

Answer: C - Fluid resuscitation is the most important initial treatment for DKA.

58. A patient presents with sudden severe headache described as "the worst headache of my life." This is most concerning for:

- A. Migraine
- B. Subarachnoid hemorrhage
- C. Tension headache

D. Cluster headache

Answer: B - "Worst headache of my life" is a classic description of subarachnoid hemorrhage.

59. Which of the following is a sign of increased intracranial pressure in an infant?

- A. Sunken fontanelle
- B. Bulging fontanelle
- C. Dry mucous membranes
- D. Tachycardia only

Answer: B - A bulging fontanelle in an infant is a sign of increased intracranial pressure.

60. A patient presents with a seizure and a blood glucose of 35 mg/dL. The most appropriate treatment is:

- A. Administer lorazepam
- B. Administer D50 IV
- C. Restrain the patient
- D. Insert an oral airway

Answer: B - Hypoglycemia-induced seizures should be treated with IV D50.

NREMT PRACTICE EXAM 1

Trauma Emergencies (Questions 61-80)

SECTION 4: Trauma Emergencies (Questions 61-80)

61. A patient presents with a penetrating chest wound that makes a sucking sound. The most appropriate treatment is:

- A. Apply a sterile dressing
- B. Apply an occlusive dressing taped on three sides
- C. Insert a chest needle
- D. Apply direct pressure

Answer: B - An occlusive dressing taped on three sides allows air to escape during exhalation.

62. The most reliable sign of a pelvic fracture is:

- A. Hip pain
- B. Crepitus on palpation
- C. Instability on lateral compression
- D. Bruising over the iliac crest

Answer: C - Pelvic instability on lateral compression is the most reliable sign.

63. A patient presents with a flail chest. The most appropriate treatment is:

- A. Apply a bulky dressing
- B. Positive pressure ventilation
- C. Stabilize the flail segment with a bulky dressing and tape
- D. Insert a chest tube

Answer: C - Stabilize the flail segment with a bulky dressing taped to the chest wall.

64. The most important sign of internal bleeding is:

- A. Visible bruising
- B. Tachycardia and hypotension
- C. Abdominal distension
- D. Pain at the injury site

Answer: B - Tachycardia and hypotension are the most important signs of significant internal bleeding.

65. A patient presents with a partial amputation of the forearm. The most appropriate treatment is:

- A. Complete the amputation
- B. Apply a tourniquet proximal to the injury
- C. Wrap the amputated part in dry gauze
- D. Apply direct pressure and splint

Answer: D - Apply direct pressure, splint, and wrap the amputated part in moist gauze on ice.

66. Which of the following is a sign of a basilar skull fracture?

- A. Periorbital ecchymosis (raccoon eyes)
- B. Unilateral pupil dilation
- C. Hypertension
- D. Tachycardia

Answer: A - Periorbital ecchymosis (raccoon eyes) and Battle's sign are signs of basilar skull fracture.

67. The most appropriate treatment for a patient with a suspected spinal injury is:

- A. Log roll the patient
- B. Apply manual in-line stabilization
- C. Sit the patient up
- D. Remove the helmet immediately

Answer: B - Manual in-line stabilization is the first step for suspected spinal injury.

68. A patient presents with a Glasgow Coma Scale of 6. This indicates:

- A. Mild brain injury
- B. Moderate brain injury
- C. Severe brain injury
- D. Normal consciousness

Answer: C - GCS $\leq$ 8 indicates severe brain injury and the need for airway management.

69. The correct treatment for a patient with a tension pneumothorax is:

- A. Needle decompression
- B. Chest seal
- C. Positive pressure ventilation

D. Pericardiocentesis

Answer: A - Needle decompression at the 2nd intercostal space, midclavicular line.

70. A patient presents with signs of hypovolemic shock. The most appropriate fluid resuscitation strategy is:

- A. Rapid infusion of 2L normal saline
- B. Small boluses to maintain SBP > 90
- C. No fluid administration
- D. D5W infusion

Answer: B - Small fluid boluses (250-500mL) to maintain SBP > 90 mmHg.

71. Which of the following is a contraindication for nasopharyngeal airway insertion?

- A. Unresponsive patient
- B. Suspected basilar skull fracture
- C. Patient with a gag reflex
- D. Pediatric patient

Answer: B - NPA is contraindicated in suspected basilar skull fracture.

72. A patient presents with a burn that is white, leathery, and painless. This is classified as:

- A. Superficial burn
- B. Partial-thickness burn
- C. Full-thickness burn
- D. First-degree burn

Answer: C - Full-thickness (third-degree) burns are white/leathery and painless.

73. The Rule of Nines for an adult assigns what percentage to the anterior trunk?

- A. 9%
- B. 18%
- C. 36%
- D. 4.5%

Answer: B - The anterior trunk is 18% of body surface area.

74. A patient presents with a chemical burn to the eye. The most appropriate treatment is:

- A. Apply neutralizing solution
- B. Irrigate with water for at least 20 minutes
- C. Cover both eyes
- D. Apply antibiotic ointment

Answer: B - Irrigate the affected eye with water for at least 20 minutes.

75. Which of the following is a sign of compartment syndrome?

- A. Pain with passive movement
- B. Warm, pink skin
- C. Bounding pulse
- D. Hypotension

Answer: A - Pain with passive movement is an early sign of compartment syndrome.

76. A patient presents with a suspected hip fracture. The most appropriate treatment is:

- A. Attempt to reduce the fracture
- B. Apply traction splint
- C. Stabilize the extremity and transport
- D. Have the patient walk to the stretcher

Answer: C - Stabilize the extremity in the position found and transport.

77. The most appropriate treatment for a patient with a suspected spinal injury who is in respiratory arrest:

- A. Jaw-thrust maneuver
- B. Head-tilt chin-lift
- C. Insert an OPA only
- D. Wait for ALS

Answer: A - The jaw-thrust maneuver is used to open the airway in suspected spinal injury patients.

78. A patient presents with a penetrating abdominal wound with protruding organs. The most appropriate treatment is:

- A. Push the organs back into the abdomen
- B. Cover with a moist, sterile dressing
- C. Apply direct pressure
- D. Apply an occlusive dressing

Answer: B - Cover protruding organs with a moist, sterile dressing. Do NOT push them back.

79. Which of the following is the most important factor in determining the severity of a burn?

- A. Color of the burn

- B. Location of the burn
- C. Total body surface area affected
- D. Age of the patient

Answer: C - Total body surface area (TBSA) affected is the most important factor.

80. A patient presents with a suspected spinal injury after a diving accident. The most appropriate immobilization is:

- A. Cervical collar only
- B. Long spine board with head blocks
- C. Sitting position
- D. Prone position

Answer: B - Full spinal immobilization with a long spine board, cervical collar, and head blocks.

NREMT PRACTICE EXAM 1

OB/Pediatrics (Questions 81-100)

SECTION 5: OB/Pediatrics (Questions 81-100)

81. A pregnant patient in her third trimester is experiencing seizures. This is most likely:

- A. Eclampsia
- B. Epilepsy
- C. Hypoglycemia
- D. Stroke

Answer: A - Eclampsia is seizures in a pregnant patient, typically after 20 weeks gestation.

82. The most appropriate position for a pregnant patient in the third trimester is:

- A. Supine
- B. Left lateral recumbent
- C. Right lateral recumbent
- D. Trendelenburg

Answer: B - Left lateral relieves pressure on the inferior vena cava.

83. A newborn has a heart rate of 80 bpm, weak cry, and blue extremities. The APGAR score for color is:

- A. 0
- B. 1
- C. 2
- D. Cannot be determined

Answer: B - Blue extremities with a pink body scores 1 for color (acrocyanosis).

84. The correct compression-to-ventilation ratio for neonatal resuscitation is:

- A. 3:1
- B. 15:2
- C. 30:2
- D. 5:1

Answer: A - Neonatal resuscitation uses a 3:1 compression-to-ventilation ratio.

85. A 2-year-old child is in respiratory failure. The most appropriate airway adjunct is:

- A. Adult OPA
- B. Pediatric OPA
- C. Endotracheal tube
- D. Nasopharyngeal airway only

Answer: B - A pediatric oropharyngeal airway is appropriate for a 2-year-old.

86. Which of the following is a sign of respiratory distress in an infant?

- A. Bradycardia
- B. Nasal flaring
- C. Hypertension
- D. Warm, dry skin

Answer: B - Nasal flaring is a sign of respiratory distress in infants.

87. A 6-month-old infant is found unresponsive and not breathing. The correct compression technique is:

- A. Two-finger technique
- B. One-hand technique
- C. Two-thumb encircling technique
- D. Heel of one hand

Answer: C - For infants, the two-thumb encircling technique is preferred for CPR.

88. The normal heart rate for a newborn is:

- A. 60-100 bpm
- B. 100-160 bpm
- C. 160-200 bpm
- D. 80-120 bpm

Answer: B - Normal newborn heart rate is 100-160 bpm.

89. A pregnant patient is experiencing vaginal bleeding in the third trimester. The most likely diagnosis is:

- A. Placenta previa
- B. Ectopic pregnancy
- C. Spontaneous abortion

D. Preeclampsia

Answer: A - Painless vaginal bleeding in the third trimester is most consistent with placenta previa.

90. The most appropriate treatment for a prolapsed umbilical cord is:

- A. Push the cord back into the uterus
- B. Place the mother in Trendelenburg and knee-chest position
- C. Apply traction to the cord
- D. Have the mother push

Answer: B - Place the mother in Trendelenburg or knee-chest position and transport immediately.

91. A 3-year-old child has a fever of 104 degF and is lethargic. The most appropriate treatment is:

- A. Administer aspirin
- B. Apply cold water immersion
- C. Administer acetaminophen and transport
- D. Bundle the child in blankets

Answer: C - Administer age-appropriate acetaminophen and transport. Do NOT use aspirin in children.

92. Which of the following is a sign of child abuse?

- A. Bruises on the shins
- B. Bruises in various stages of healing
- C. Scraped knees
- D. Forehead bump

Answer: B - Bruises in various stages of healing may indicate child abuse.

93. A 10-year-old child is in anaphylaxis. The correct dose of epinephrine auto-injector is:

- A. 0.15 mg IM
- B. 0.3 mg IM
- C. 0.5 mg IM
- D. 1.0 mg IV

Answer: B - For children >30 kg, the adult dose of 0.3 mg IM is appropriate.

94. The most common cause of cardiac arrest in children is:

- A. Heart disease
- B. Respiratory failure
- C. Trauma
- D. Poisoning

Answer: B - Respiratory failure is the most common cause of cardiac arrest in children.

95. A newborn is delivered and is not breathing. The first step is:

- A. Suction the mouth and nose
- B. Dry, warm, and stimulate
- C. Start chest compressions
- D. Administer oxygen

Answer: B - Dry, warm, and stimulate the newborn first.

96. Which of the following is a sign of dehydration in an infant?

- A. Bulging fontanelle
- B. Tears when crying
- C. Sunken fontanelle
- D. Wet diapers every hour

Answer: C - A sunken fontanelle is a sign of dehydration in infants.

97. A 5-year-old child has a suspected fracture of the forearm. The most appropriate treatment is:

- A. Attempt to reduce the fracture
- B. Apply a traction splint
- C. Splint in position found and transport
- D. Apply heat to the area

Answer: C - Splint the extremity in the position found and transport.

98. The correct dose of oral glucose for a pediatric patient is:

- A. Full adult dose (15-20g)
- B. Half the adult dose (7-10g)
- C. Quarter dose (3-5g)
- D. No oral glucose for pediatrics

Answer: B - Pediatric patients should receive half the adult dose of oral glucose.

99. A pregnant patient is in active labor. The second stage of labor is defined as:

- A. Onset of contractions to full dilation

- B. Full dilation to delivery of the baby
- C. Delivery of the baby to delivery of the placenta
- D. Onset of contractions to delivery of the placenta

Answer: B - The second stage of labor is from full cervical dilation to delivery of the baby.

100. Which of the following is a sign of impending delivery?

- A. Contractions every 10 minutes
- B. Mother feels the urge to push
- C. Bloody show
- D. Rupture of membranes only

Answer: B - The urge to push (bearing down) is a sign of impending delivery.

NREMT PRACTICE EXAM 1

Operations (Questions 101-120)

SECTION 6: Operations (Questions 101-120)

101. The minimum number of personnel required to lift a patient on a long spine board is:

- A. 2
- B. 3
- C. 4
- D. 5

Answer: C - A minimum of 4 personnel is recommended for safe spinal immobilization.

102. Which of the following is a requirement for informed consent?

- A. The patient must be over 18
- B. The patient must understand the risks and benefits
- C. The patient must sign a form
- D. A witness must be present

Answer: B - Informed consent requires that the patient understands the risks, benefits, and alternatives.

103. A patient refuses treatment. The most appropriate action is:

- A. Treat the patient anyway
- B. Document the refusal and have the patient sign
- C. Restrain the patient
- D. Call the police

Answer: B - Document the refusal and have the patient sign a refusal form.

104. Which of the following is a sign of a hazardous materials incident?

- A. Multiple patients with similar symptoms
- B. A single patient with chest pain
- C. A patient with a broken leg
- D. A patient with a headache

Answer: A - Multiple patients with similar symptoms should raise suspicion of a hazmat incident.

105. The correct approach to a scene with a potential hazmat incident is:

- A. Enter the scene quickly to rescue patients
- B. Stay upwind and uphill, and call for specialized help
- C. Begin decontamination immediately
- D. Ignore the hazmat and focus on patient care

Answer: B - Stay upwind, uphill, and at a safe distance. Call for specialized hazmat teams.

106. Which of the following is the most important piece of information to provide to the receiving hospital?

- A. Patients insurance information
- B. Patients name and age
- C. Chief complaint, vital signs, and interventions
- D. Time of arrival at the scene

Answer: C - The most important information is the patient's chief complaint, vital signs, and interventions.

107. The correct order of operations at a motor vehicle collision scene is:

- A. Scene size-up, primary assessment, history, secondary assessment
- B. Primary assessment, scene size-up, history, secondary assessment
- C. History, primary assessment, scene size-up, secondary assessment
- D. Secondary assessment, primary assessment, scene size-up, history

Answer: A - Scene size-up is always first.

108. Which of the following is a sign of a potentially violent scene?

- A. Calm bystanders
- B. Loud music
- C. Aggressive behavior from bystanders
- D. A single patient

Answer: C - Aggressive behavior from bystanders is a sign of a potentially violent scene.

109. The most appropriate action when arriving at a scene with downed power lines is:

- A. Approach the scene carefully
- B. Stay at least 50 feet away and call the power company
- C. Use a wooden stick to move the power lines

D. Drive over the power lines quickly

Answer: B - Stay at least 50 feet away from downed power lines.

110. The correct method for lifting a patient is:

- A. Bend at the waist and lift with your back
- B. Bend at the knees and lift with your legs
- C. Lift with one arm
- D. Twist while lifting

Answer: B - Always bend at the knees and lift with your legs.

111. A patient is experiencing a behavioral emergency. The most appropriate approach is:

- A. Restrain the patient immediately
- B. Use a calm, reassuring voice and maintain a safe distance
- C. Threaten the patient with consequences
- D. Ignore the patient's behavior

Answer: B - Use a calm, reassuring voice and maintain a safe distance.

112. Which of the following is a sign of a patient who is a danger to themselves or others?

- A. Calm demeanor
- B. Cooperative behavior
- C. Threats of violence
- D. Normal speech

Answer: C - Threats of violence indicate a patient may be a danger.

113. The most appropriate action when a patient has a communicable disease is:

- A. Refuse to treat the patient
- B. Use standard precautions and treat the patient
- C. Wear a hazmat suit
- D. Isolate the patient and leave

Answer: B - Use standard precautions and treat the patient.

114. Which of the following is the correct documentation practice?

- A. Document only abnormal findings
- B. Document everything objectively
- C. Document only what the patient says
- D. Document only your interventions

Answer: B - Document everything objectively.

115. A patient presents with a DNR order. The most appropriate action is:

- A. Begin CPR immediately
- B. Honor the DNR and provide comfort care only
- C. Contact medical control for guidance
- D. Ask the family to revoke the DNR

Answer: C - Verify the DNR is valid and contact medical control for guidance.

116. Which of the following is the most important factor in determining the priority of patients at a mass casualty incident?

- A. Age of the patient
- B. Severity of injuries
- C. Social status
- D. Insurance coverage

Answer: B - Severity of injuries determines patient priority using START triage.

117. The START triage system categorizes patients into how many categories?

- A. 2
- B. 3
- C. 4
- D. 5

Answer: C - START triage: Immediate (Red), Delayed (Yellow), Minor (Green), Deceased (Black).

118. A patient who can walk at a mass casualty incident is initially categorized as:

- A. Immediate (Red)
- B. Delayed (Yellow)
- C. Minor (Green)
- D. Deceased (Black)

Answer: C - Patients who can walk are categorized as Minor (Green).

119. Which of the following is the correct method for communicating with a hearing-impaired patient?

- A. Shout loudly

- B. Use written notes or sign language
- C. Speak very slowly
- D. Ignore the patient

Answer: B - Use written notes, sign language, or other visual communication methods.

120. The most important principle of EMS documentation is:

- A. Use medical jargon
- B. Document only what you did
- C. If it wasn't documented, it wasn't done
- D. Document only abnormal findings

Answer: C - The golden rule: "If it wasn't documented, it wasn't done."

DISCLAIMER: These practice questions are for educational purposes only. They are not official NREMT exam questions. Always verify answers against current NREMT guidelines and your course materials.